

Amendment No. 1 to HB1203

Terry
Signature of Sponsor

AMEND Senate Bill No. 359*

House Bill No. 1203

by deleting all language after the enacting clause and substituting:

SECTION 1. Tennessee Code Annotated, Section 63-6-106, is amended by designating the existing language as subsection (a) and adding the following as a new subsection:

(b)

(1) During an informal initial application interview during an executive session held pursuant to § 63-1-169, the board may, in its discretion, issue a license under a private advocacy order requiring the applicant to maintain advocacy of a peer assistance program approved by the board. Any determination regarding the necessity for a private advocacy order must be individually tailored to the applicant's condition and how it currently impairs the applicant's ability to practice medicine in a competent, ethical, and professional manner absent such advocacy. A private advocacy order is not a restriction on the license.

(2) For an individual practicing under a private advocacy order, failure to maintain the advocacy of the peer assistance program constitutes a violation of the practice act for which the board may take disciplinary action.

(3) Private advocacy orders are confidential, privileged, and not public records subject to inspection by citizens of this state, as described in §§ 8-44-104(a) and 10-7-503. However, if a practitioner fails to maintain advocacy and formal disciplinary proceedings are initiated as a result of that failure, then the private advocacy order becomes a public record.

SECTION 2. Tennessee Code Annotated, Section 63-9-118, is amended by designating the existing language as subsection (a) and adding the following as a new subsection:

(b)

(1) During an informal initial application interview during an executive session held pursuant to § 63-1-169, the board may, in its discretion, issue a license under a private advocacy order requiring the applicant to maintain advocacy of a peer assistance program approved by the board. Any determination regarding the necessity for a private advocacy order must be individually tailored to the applicant's condition and how it currently impairs the applicant's ability to practice medicine in a competent, ethical, and professional manner absent such advocacy. A private advocacy order is not a restriction on the license.

(2) For an individual practicing under a private advocacy order, failure to maintain the advocacy of the peer assistance program constitutes a violation of the practice act for which the board may take disciplinary action.

(3) Private advocacy orders are confidential, privileged, and not public records subject to inspection by citizens of this state, as described in §§ 8-44-104(a) and 10-7-503. However, if a practitioner fails to maintain advocacy and formal disciplinary proceedings are initiated as a result of that failure, then the private advocacy order becomes a public record.

SECTION 3. This act takes effect upon becoming a law, the public welfare requiring it.